False, indisputable | False, because Gie City lied/withheld information

False/inaccurate, disputable | Subjective - projecting their values onto Gie City's life

ACT Government | Canberra Health Services

9 February, 2024

Gungahlin Family Healthcare, Dr.
Gungahlin Family Healthcare
2 Franz Bormann Close
Gungahlin ACT 2912

Patient: **Giles Cullity**
Date of Birth: **22/7/1993**
Date of Visit: **9/2/2024**

Dear Dr. Gungahlin Family Healthcare:

Thank you for your ongoing care of Mr Giles Cullity. I reviewed him today for the final time at the City CRS; as his mental state has been stable, we are referring him back to primary care. I have recommended he continues Aripiprazole 15mg mane long-term. He is aware of early warning signs and how to seek re-referral.

Below are my notes for this consultation.

If you have questions, please do not hesitate to call me. I look forward to following your patient along with you.

Sincerely,

Dr Pieter Hurter
Psychiatrist
CCHC CRS LOCUM CLINIC

CC: No Recipients

Situation:
Outpatient review at City CRS on 09 February 2024 at 13:30.

Present:
- Client: Giles Cullity
- Dr Pieter Hurter (Psychiatrist)

Problems:
Schizoaffective disorder (differential: bipolar disorder).

Legal status:
Voluntary

Medication:
1. Aripiprazole 15mg mane
2. Quetiapine 12.5-25 mg nocte
(Past history of Clozapine treatment until 2022)

There is a **history of long QTc** which will impact future medication decisions.

GP:
Dr Ramchander - Gungahlin Family Health Services

Background:
Giles is a 30-year-old gentleman with a lived experience of schizoaffective disorder. He lives in shared housing, and his parents live locally. He is financially supported by the DSP and he has NDIS in place, although he is not currently utilising this. He has previously been treated with Clozapine (which was commenced in Adelaide), but self-ceased this in around August 2022 due to concerns about weight gain.

There is a strong family history of BPAD, and a younger brother is documented to have BPAD and ASD; there is also a history of paternal grandfather, paternal uncle, and maternal uncle with reported BPAD.

Giles has a forensic history of allegedly assaulting police in 2010 and alleged attempted murder in 2013: reportedly, whilst homeless, Giles argued with another homeless person over the ownership of a mattress. He then cut the neck of the other person. Received 3-year suspended sentence and maintained on home detention bail with monitoring device on the ankle. Concluded in December 2017.

Giles has been doing well in recent months and his care with the City CRS has been stepped down from GC to CEC.

Review:
Giles presented on time for appointment. Giles reports he has been doing well since the last review, with no new concerns. He exercises almost every day - although in recent weeks he has developed "shin splints", which have reduced his ability to run - he has been doing interval sprinting. He reports his home life is stable, and he has good relationships with his house mates. He enjoys listening to podcasts - Huberman Lab in particular, and has found benefit from this. He reports he also enjoys reading on self-development.

He talks to his parents on the phone once or twice a month; reports they are on good terms, but not overly close. He reports he doesn't require any other supports - an NDIS planning meeting is scheduled for the next month, but he does not feel he needs any input from NDIS, as he is able to attend to all his own needs independently.

Giles has no mental health concerns; initially he reported that his mood has been stable, with no significant changes. He enjoys activities, and there is no loss of pleasure. No evidence of any depression or mood

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elevation. He reports his sleep schedule is regular, going to sleep at 10-11pm and getting up at 5 am. No insomnia. He uses mindfulness and body scan at night. No changes in appetite or weight - although Giles reported a goal of developing more muscle. His energy is good - which he uses to exercise (nearly) every day - doing weights and jogging.

Giles denied any psychotic symptoms on screening. Explored the topics from last review - designing gym equipment; Giles reports he has not spent much time thinking about this, but does still have an interest in this.

Medication: is taking Aripiprazole 15mg in the morning. Identifies lethargy, but no other side effects.

Interview focused on early warning signs; Giles identified:
- "If I have unrealistic thoughts"
- "A general feeling of excitement, that doesn't go away"
- "Lack of focus, getting distracted by driven thoughts"

Physical health:
No concerns; is due to have bloods done as requested by his GP.

AOD:
Giles reports healthy habits, with no alcohol or other recreational substance use.

MSE:
Giles presented as a muscular man of stated age with neatly trimmed beard. He was casually dressed in clean clothes. His physical appearance was not neglected. No involuntary movements noted.
Speech - NAD, not pressured.
His affect was assessed as euthymic, with good reactivity.
He stated his mood was "pretty good".
No FTD evident.
Content:
- No delusional content evident today.
- No suicidal ideation.
- No risks to others identified.

Fair insight.

Impression:
Giles presents as stable in his mental state today, with no evidence of a mood disturbance and no psychotic symptoms. He was able to identify early warning signs and appropriate actions. No risks to self or others identified from today's assessment. As Giles has been stable, his care can be transferred to primary care.

Plan:
1. Highlighted the importance of continuing with Aripiprazole 15mg daily in the long term.
2. Encouraged utilising NDIS, even though he does not see any need.

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3. Discussed early warning signs and how to seek medical care.
4. Discussed discharge from clinic and continuing care with GP, which Giles was happy with.
5. Letter to GP.
6. Closure from City CRS.

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